JOEL EMBIID

2026-2027 Significant Injury Risk Forecast

A transparent, public-data sports-risk calculation based on injury history, workload, recurrence, and comparable NBA centers

REPORT TYPE	Public-data sports-risk analysis
ANALYSIS DATE	July 18, 2026
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Disclaimer. This report is a statistical sports-risk estimate based solely on publicly available information. It is not a medical diagnosis, betting recommendation, or statement about confidential health information.

The uncertainty interval is a model-sensitivity range, not a clinical confidence interval.

Executive summary

68%	31%	55%-80%
At least one significant basketball injury	At least 30 total injury-missed games	Reasonable model-sensitivity interval

Relative classification Very high versus a typical NBA player; high to very high versus older, high-usage centers.	Estimate certainty 61 / 100
Hallucination risk Low for cited historical facts; moderate for the forecast because matched-cohort and medical data are incomplete.	Most consequential workload variable Total competitive exposure, especially minutes per game.

The central estimate is produced by a four-model ensemble: a comparable-center proxy prior, a literature-anchored workload model, a player-specific recurrent-event estimate, and a Bayesian shrinkage calculation. The ensemble preserves the supplied analysis and does not claim access to nonpublic medical information.

Principal risk drivers

- Nine qualifying seasons in ten, the strongest player-specific signal.
- Repeated left-knee involvement, including documented procedures and recurrent symptoms.
- Recent availability: 39, 19, and 38 regular-season games across the three most recent seasons in the supplied dataset.
- Multiple prior operations and severe episodes, while recognizing that contact injuries and recurrent lower-extremity injuries are not equivalent.
- Future court exposure, with minutes per game used as the clearest published numerical workload factor found in the cited literature.

Headline interpretation

The model says that a qualifying injury is more likely than not. It does not predict the timing, anatomical location, or certainty of an injury, and it does not establish that exceptional height independently causes injury.

1. Scope and event definition

The modeled event is at least one basketball-related injury during the 2026-27 regular season or playoffs that satisfies one or more of the following criteria:

Included	Excluded
At least 15 consecutive calendar days unavailable.	Ordinary rest or planned load management without an identified injury.
At least 10 NBA games missed from a single injury episode.	Suspensions, personal absences, or coaching decisions.
Surgery or a season-ending designation.	Illnesses and non-basketball medical events, including the supplied analysis's exclusion of appendicitis and the April 2026 appendectomy.
A medically reported recurrence of a prior major lower-extremity injury causing at least 10 missed games.	All-cause games not played treated automatically as injury games.

NBA injury reports distinguish injuries, illnesses, rest, and load management, although older public reporting does not always allow perfect retrospective separation. [2]

Primary outputs

Outcome 1: Probability of at least one qualifying significant injury.

Outcome 2: Probability of at least 30 total games missed because of injury.

The second outcome is more uncertain because historical all-cause absence can include illness, rest, rehabilitation management, or other non-injury absences.

2. Data and source methodology

Participation and season totals were taken from the supplied analysis, which cites the NBA player profile and Basketball Reference. [1] [3] Injury episodes and procedures were linked to official NBA reports where available. [4]-[14]

- Games played, minutes per game, and playoff appearances are treated as measured participation data.
- Approximate season minutes are reconstructed as games played multiplied by minutes per game unless a directly reported total was supplied.
- A season's 'games not played' is an all-cause count and is not automatically equated with injury games.
- Back-to-back participation could not be reconstructed consistently across all ten seasons from the public data used in the supplied analysis.
- Diagnosis-level detail, complete rehabilitation status, and confidential medical information are unavailable.

No new medical diagnosis, causal claim, or regression coefficient has been introduced in formatting this report.

3. Ten-season Joel Embiid history

Embiid is listed at 7 feet and 270 pounds and will enter the modeled season at age 32. The supplied dataset records 38 regular-season games in 2025-26, following 19 in 2024-25 and 39 in 2023-24. [1] [3]

Legend: YES = meets the report's significant-injury definition; NO = below the defined threshold. † denotes a documented surgery or invasive procedure. K denotes left-knee involvement. EXCL denotes a non-basketball medical event excluded from the model.

Season	Age	RS G / MPG	Approx. minutes	Not played	PO G / MPG	Main documented episode	Flag
2016-17	22	31 / 25.4	787	51	-	Torn left meniscus; surgery; season ended. [4]	YES † K
2017-18	23	63 / 30.3	1,909	19	8 / 34.8	Left orbital fracture and concussion; surgery; about three weeks and 10 games. [5]	YES †
2018-19	24	64 / 33.7	2,157	18	11 / 30.4	Left-knee soreness; MRI reported no structural damage; multiweek absence/load management. [6]	YES K
2019-20	25	51 / 29.5	1,504	22 of 73	4 / 36.3	Torn ligament in left hand; surgery; returned after about three weeks. [7]	YES †
2020-21	26	51 / 31.1	1,586	21 of 72	11 / 32.5	Left-knee bone bruise; unavailable at least two weeks. [8]	YES K
2021-22	27	68 / 33.8	2,298	14	10 / 38.5	Right orbital fracture/concussion; right-thumb ligament tear; later thumb and finger procedures. [9]	YES †
2022-23	28	66 / 34.6	2,284	16	9 / 37.3	Right-knee sprain; missed two playoff games, below the defined threshold. [10]	NO
2023-24	29	39 / 33.6	1,310	43	6 / 41.3	Left lateral-meniscus injury and procedure. [11]	YES † K
2024-25	30	19 / 30.2	574	63	-	Ongoing left-knee problems; medically shut down; arthroscopic surgery. [12]	YES † K
2025-26	31	38 / 31.6	1,201	44	7 / 33.3	Right-oblique strain; later ankle/hip issues. Appendectomy excluded. [13] [14]	YES EXCL

Historical player-specific result

x = 9 significant-injury seasons out of n = 10.

The model does not include Embiid's first two NBA seasons, both missed because of right-foot injuries. Including those seasons would increase, rather than decrease, the historical risk signal. [3]

The 2025-26 appendicitis and appendectomy are excluded because they were non-basketball medical events. The supplied analysis also reports that he did not play both games of any 2025-26 back-to-back; this was not reconstructed independently for every season.

4. Comparable-center cohort

The cohort emphasizes size, substantial NBA responsibility, and sufficient career history. It is a screening framework rather than a medically audited matched dataset.

Player	Listed size	Cohort role
Shaquille O'Neal	7'1", 325 lb	Extreme-size, high-usage comparison. [15]
Yao Ming	7'6", 310 lb	Extreme-height and injury-survivorship comparison. [16]
Dwight Howard	6'10", 265 lb	High-minute, long-career power center. [17]
Marc Gasol	6'11", 255 lb	Older high-responsibility center. [18]
Brook Lopez	7'1", approx. 280 lb	Tall-center longevity comparison. [19]
DeMarcus Cousins	6'10", 270 lb	High-usage center with major late-career injury burden. [20]

Cohort layers

- Broad tall-center cohort: all six players.
- High-usage cohort: O'Neal, Ming, Howard, and Cousins.
- Closest size/risk-pattern subgroup: O'Neal, Ming, and Cousins.

Porzingis-type players were not treated as primary matches because of different body mass and offensive workload. Younger players such as Victor Wembanyama and Chet Holmgren do not yet supply the requested age-28-plus history.

Important cohort limitation

There is no complete, publicly accessible, medically audited database from which the supplied analysis could calculate an exact rate for '10 games or 15 days missed' among matched very tall centers. Treating every absence in box-score records as injury would mix injury, illness, rest, and coaching decisions.

Published all-player season-ending-injury benchmark	15.6%
Broader comparable-center modeling prior	42%
Prior sensitivity range	30%-55%

The 42% value is an explicit analyst-selected proxy prior, not a published matched-cohort statistic. It is higher than the 15.6% season-ending benchmark because the modeled event is broader and the target population is older, high-minute, and previously injured. The cited study identified minutes per game as its strongest measured season-ending-injury predictor. [21]

This approach partially addresses survivorship bias by retaining injury-shortened careers such as Ming and Cousins, but it cannot eliminate survivorship bias or fully adjust for era, pace, medical treatment, and schedule differences.

Does extreme height itself increase the estimate?

Not directly. A large NBA epidemiology study found that lower-extremity injuries were common, while guards showed the highest injury ratios by position. The greatest percentages occurred among players logging 26-35 minutes per game and players with 6-15 years of experience. That evidence does not justify a causal penalty merely for being seven feet tall. [22]

Height and weight are therefore used for comparator selection. Their fitted coefficients are set to zero in the numerical model because defensible independent estimates were unavailable.

5. Risk variables

The requested variables are retained below. Their status is separated into measured, approximated, or unavailable to prevent assumptions from being presented as established facts.

Var	Meaning	Status / source	Model treatment
A	Age entering season	Measured from public profile and season record.	Context only; coefficient set to zero in Model B.
H	Height	Measured listed height.	Comparator selection; coefficient set to zero.
W	Listed weight or BMI proxy	Measured listed weight; BMI not used.	Comparator selection; coefficient set to zero.
G	Games played, prior three seasons	Measured participation data.	Captured indirectly through durability history.
M	Minutes / workload	Measured or reconstructed from games × MPG.	Published per-minute odds ratio used in Model B.
I	Number and severity of prior significant injuries	Classified from public reports.	Primary input to recurrent-event and Bayesian models.
R	Same-region recurrence	Approximated from documented body-region history.	Sensitivity analysis; no invented logistic coefficient.
S	Publicly confirmed procedures	Measured where officially reported.	Included qualitatively and through event history.
L	Recent workload and playoff load	Measured or scenario-assigned.	Used in scenario exposure calculation.
D	Recent durability trend	Measured games played and absence history.	Captured through player-specific history.
C	Publicly confirmed chronic/degenerative indicator	Unavailable as a reliable numerical measure.	No undisclosed diagnosis inferred; coefficient omitted.

The model is intentionally conservative about coefficient construction: variables without defensible published numerical estimates are not assigned invented regression weights.

6. Statistical ensemble

The final estimate combines four models with fixed weights. Published quantities, analyst-selected assumptions, and player-specific history are labeled separately.

Methodology flow

Model A
Comparable-center proxy prior
15% weight

→

Model B
Literature-anchored workload model
15% weight

→

Model C
Player-specific recurrent event
35% weight

→

Model D
Bayesian shrinkage
35% weight

Figure 1. Four-model ensemble. Model weights are analyst-selected and sum to 1.00.

6. Statistical ensemble - models and arithmetic

Model A: Comparable-center prior

Analyst-selected proxy prior:

$$p_A = 0.42$$

Sensitivity: $p_A \in [0.30, 0.55]$.

This proxy is necessitated by incomplete injury-specific cohort data. It is not presented as a published matched-center rate.

Model B: Literature-anchored logistic calculation

The cited season-ending-injury study reported an odds ratio of approximately 1.06 for each additional minute per game. The calculation uses that published quantity without inventing coefficients for age, height, weight, recurrence, or other requested predictors. [21]

Baseline calibration:

$$\mathrm{logit}(p_B) = \mathrm{logit}(0.42) + \ln(1.06)(M - 30)$$

For the normal-load assumption M = 33:

$$\mathrm{logit}(0.42) = -0.3228$$
$$\ln(1.06) = 0.05827$$
$$\mathrm{logit}(p_B) = -0.3228 + 0.05827(3) = -0.1480$$
$$p_B = e^{-0.1480} / (1 + e^{-0.1480}) = 0.463$$

Coefficients not numerically supported by defensible estimates were set to zero inside Model B:

$$\beta_A = \beta_H = \beta_W = \beta_G = \beta_I = \beta_R = \beta_S = \beta_D = 0$$

This does not imply that those variables are biologically irrelevant. It prevents fabricated coefficients from entering the calculation.

Published evidence versus analyst choices

Published numerical evidence	Approximate odds ratio of 1.06 per additional minute per game in the cited season-ending-injury study.
Analyst-selected calibration	42% baseline at 30 minutes per game.
Intentionally omitted coefficients	Age, height, weight, prior games, recurrence, surgery, and durability coefficients were not invented.

6. Statistical ensemble - player-specific models

Model C: Player-specific recurrent-event estimate

With a uniform Beta prior:

$$p_C \sim \mathrm{Beta}(1, 1)$$

After nine event seasons and one nonevent season:

$$p_C \mid \text{history} \sim \mathrm{Beta}(10, 2)$$

Posterior mean:

$$p_C = 10 / 12 = 0.833$$

This estimate is not used alone because seasons are not independent, event classification is imperfect, and several facial or hand injuries were contact events rather than recurrent durability failures.

Model D: Bayesian shrinkage

Use the 42% comparator prior with an effective prior sample size of six seasons:

$$\alpha_0 = 6(0.42) = 2.52 \quad \beta_0 = 6(0.58) = 3.48$$

After observing nine significant seasons in ten:

$$p_D = (2.52 + 9) / (2.52 + 3.48 + 10) = 11.52 / 16 = 0.720$$

Final weighted ensemble

Fixed weights:

$$w_A = 0.15,\ w_B = 0.15,\ w_C = 0.35,\ w_D = 0.35$$

Player-specific models receive more weight because Embiid has ten observable seasons and repeated documented injuries. They do not receive all the weight because injury mechanisms differ and event classification is imperfect.

$$p_{\text{ensemble}} = 0.15(0.420) + 0.15(0.463) + 0.35(0.833) + 0.35(0.720) = 0.676$$

CENTRAL ESTIMATE

$$\mathbf{p_{ensemble} \approx 68\%}$$

The reported reasonable range of 55%-80% is a model-sensitivity interval reflecting alternative priors, recurrence weighting, workload exposure, and unresolved data limitations. It is not a clinical confidence interval.

7. Workload scenarios

The normal scenario is anchored at 64 regular-season games at 33 minutes per game, plus eight playoff games at 36 minutes.

Scenario exposure:

$$E_s = G_{RS}M_{RS} + G_{PO}M_{PO}$$

Probability adjustment:

$$p_s = 1 - (1 - 0.676)^{E_s/E_{normal}}$$

This is a simplifying exposure calculation. It assumes that injury opportunity scales approximately with court exposure; it does not prove that every additional minute causes injury.

Scenario	RS games	RS MPG	Playoff exposure	Total modeled minutes	Estimated probability
Managed	58	29.5	6 games × 34 min	1,915	59%
Normal starter	64	33.0	8 games × 36 min	2,400	68%
Heavy competitive	72	35.5	12 games × 38 min	3,012	76%

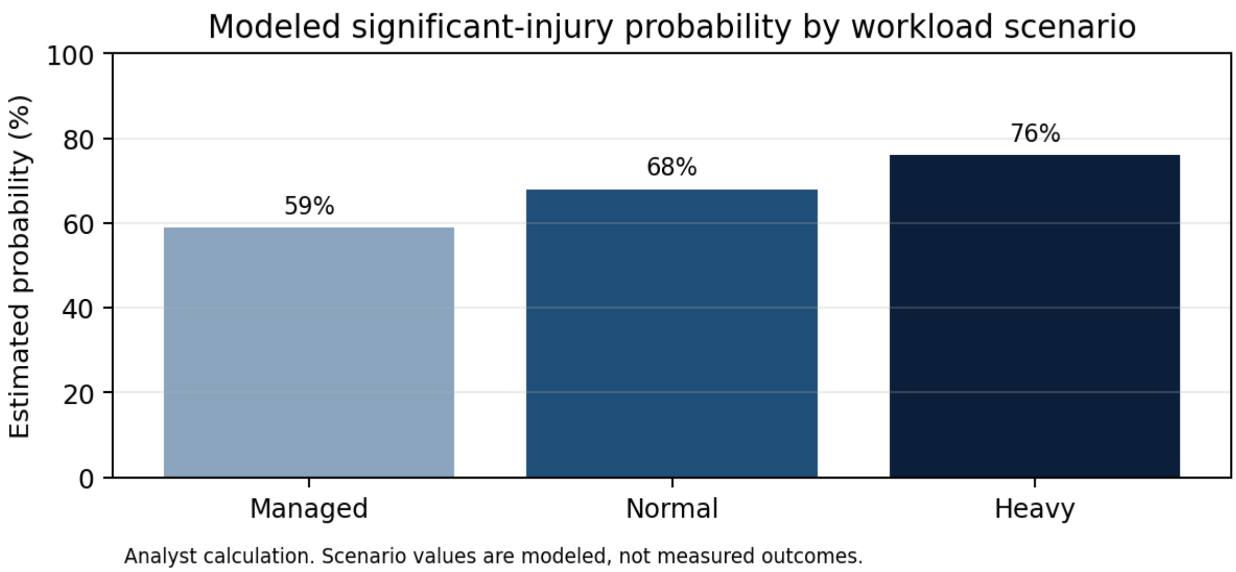

Figure 2. Workload-scenario comparison. Values are analyst calculations based on the supplied exposure formula.

A cited 2026 study using nine seasons of audited NBA medical records found no significant reduction in subsequent injury among players who missed more games for rest or load management, including analyses restricted to stars and players with prior injuries. The managed scenario therefore receives only a moderate reduction rather than assuming that sitting back-to-backs eliminates underlying risk. [23]

8. Games-missed probabilities

Public records do not perfectly distinguish injury from rest in every historical absence. The severity model therefore uses games not played only as a proxy after first establishing that a significant injury occurred.

Among the nine significant-injury seasons in the supplied classification:

- All nine involved at least 10 total games not played.
- Six involved at least 20.
- Four involved at least 30.
- Three clearly support at least 41 injury-dominated games after discounting the 2025-26 appendectomy.

Using Laplace smoothing:

$$P(K \mid \text{significant injury}) = (x_K + 1) / (9 + 2)$$

Then:

$$P(K) = 0.676 \times P(K \mid \text{significant injury})$$

Threshold	Calculation	Estimated probability	Reasonable range
At least 10 games	0.676(10/11)	61%	49%-73%
At least 20 games	0.676(7/11)	43%	31%-57%
At least 30 games	0.676(5/11)	31%	20%-47%
At least 41 games	0.676(4/11)	25%	15%-39%

These threshold estimates are more uncertain than the headline event probability. Historical 'games not played' can contain rest, illness, rehabilitation management, or other absences and is not automatically identical to injury games.

9. Sensitivity analysis

Change	Revised estimate
Broad cohort prior at 30%	62%
Central cohort prior at 42%	68%
Narrow/high-risk prior at 55%	73%
Recent three seasons given double weight	69%
Height and weight removed	No material change
Recurrent-knee evidence at half strength	64%
Recurrent-knee evidence at double strength	72%
Exposure reduced 10%	64%
Exposure reduced 20%	59%
Exposure reduced 30%	55%

9. Sensitivity analysis - interpretation

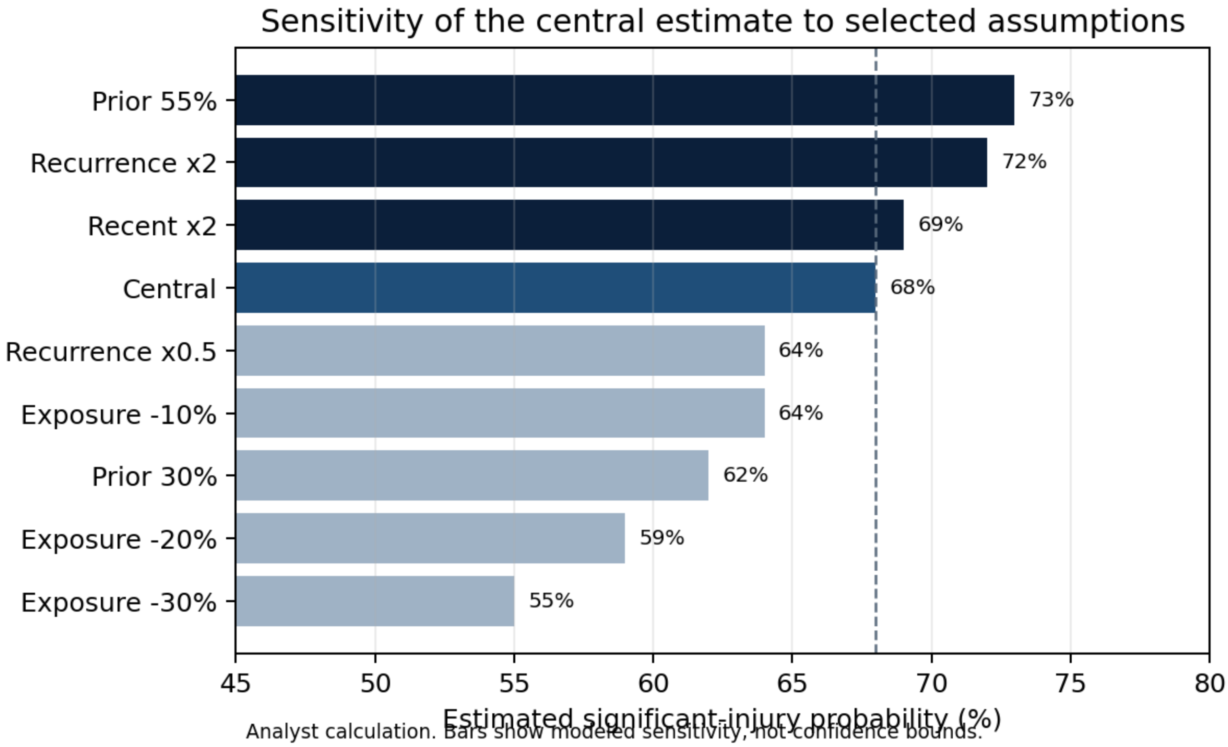

Figure 3. Selected sensitivity results. The largest modeled movement comes from reducing total exposure or shifting the comparator prior.

Alternative single-episode missed-game definitions

Definition	Estimated risk
At least 5 games	79%
At least 10 games	71%
At least 15 games	63%
At least 20 games	55%

Largest sources of uncertainty

- No publicly accessible, medically audited, diagnosis-level dataset for Embiid and a matched center cohort.
- Imperfect retrospective separation of injury, illness, rest, rehabilitation management, and healthy absence.
- Small player-specific sample: ten modeled seasons, with injury mechanisms that are not homogeneous.
- Exposure model assumes approximate scaling with minutes and games; it is not a causal biomechanical model.
- The 42% comparable-center prior and ensemble weights are transparent analyst choices rather than published coefficients.

The dominant uncertainty is data classification and calibration, not arithmetic.

10. Interpretation and limitations

What the model estimates

The report estimates the probability that at least one qualifying basketball-related injury event will occur during the 2026-27 regular season or playoffs, using public participation history, documented injury episodes, a proxy comparable-center prior, workload evidence, and player-specific recurrence history.

What the model cannot establish

- It cannot diagnose a current condition or determine whether an undisclosed condition exists.
- It cannot predict the exact timing, body part, severity, treatment, or recovery course of a future injury.
- It cannot prove that exceptional height independently causes injury.
- It cannot establish that a specific load-management schedule prevents injury.
- It cannot convert all-cause games missed into exact injury-only absences with full confidence.

Contact injuries versus recurrent lower-extremity conditions

The orbital fractures and hand injury were contact-related events and should not be interpreted as equivalent to recurring knee or lower-extremity problems. They contribute to the broad significant-injury event history but provide less direct evidence about future knee deterioration. This is one reason the recurrent-event estimate is not used alone.

What could move the estimate downward

- Entering training camp without publicly reported restrictions.
- Tolerating regular practices and playing consistently through the first 20-25 games.
- Avoiding recurrent knee symptoms or repeated lower-extremity absences.
- A sustained lower-exposure role without compensating increases in game intensity.

What could move the estimate upward

- An official preseason restriction or delayed ramp-up.
- Repeated knee-related absences or escalating lower-extremity symptoms.
- Another publicly confirmed invasive procedure.
- A heavy competitive workload with elevated minutes and extended playoff exposure.

The estimate is best understood as a transparent risk framework that can be updated when new public evidence becomes available.

11. Final conclusion

P(significant injury in 2026-27)

68%

Reasonable interval: 55%-80%

Probability of at least 30 injury-missed games	31%
Relative classification	Very high versus a typical NBA player; high to very high versus older, high-usage centers.
Principal risk drivers	Nine qualifying seasons in ten; repeated left-knee involvement; low recent availability; multiple prior procedures; future court exposure.
Most consequential workload variable	Total competitive exposure, especially minutes per game.
Estimate certainty	61 / 100
Hallucination risk	Low for cited historical facts; moderate for the forecast.

The model's principal signal is Embiid's own history, particularly repeated left-knee involvement and extremely limited recent availability. Height alone is not treated as a causal risk factor. Reducing total competitive exposure produces the largest controllable modeled reduction, but current research does not establish that isolated rest days reliably prevent injury.

This report should be independently reviewed before being used for any decision-making purpose.

Sources and notes

All links below are clickable. Access date: July 18, 2026. Source numbering corresponds to citations in the report.

Official NBA and team sources

1. NBA player profile: Joel Embiid
https://www.nba.com/player/203954/joel-embiid
2. NBA Injury Report, 2025-26 season
https://official.nba.com/nba-injury-report-2025-26-season/
4. Embiid has surgery to repair torn meniscus
https://www.nba.com/news/philadelphia-76ers-joel-embiid-has-surgery-repair-torn-meniscus
5. Embiid undergoes surgery for fractured orbital bone
https://www.nba.com/news/sixers-joel-embiid-undergoes-surgery-fractured-orbital-bone
6. Official release: Embiid left-knee soreness
https://www.nba.com/news/joel-embiid-left-knee-soreness-official-release
7. Embiid returns after torn hand ligament
https://www.nba.com/news/joel-embiid-returns-torn-ligament-hand
8. Knee injury forces Embiid exit versus Wizards
https://www.nba.com/news/knee-injury-forces-joel-embiid-exit-vs-wizards
9. Embiid out with right orbital fracture and concussion
https://www.nba.com/news/76ers-center-joel-embiid-out-indefinitely-with-right-orbital-fracture-concussion
10. Embiid to miss playoff game with sprained knee
https://www.nba.com/news/embiid-to-miss-game-4-with-sprained-knee-leonard-knee-sidelined-game-4-vs-suns
11. Embiid procedure on left knee
https://www.nba.com/news/joel-embiid-procedure-left-knee
12. Embiid arthroscopic surgery on left knee
https://www.nba.com/news/sixers-joel-embiid-arthroscopic-surgery-left-knee
13. Embiid to miss games with strained right oblique
https://www.nba.com/news/joel-embiid-to-miss-at-least-3-games-with-a-strained-right-oblique
14. Embiid out following appendicitis surgery
https://www.nba.com/news/sixers-joel-embiid-out-appendicitis-surgery

Player participation and statistical records

3. Basketball Reference: Joel Embiid
https://www.basketball-reference.com/players/e/embiijo01.html
15. Basketball Reference: Shaquille O'Neal
https://www.basketball-reference.com/players/o/onealsh01.html
16. Basketball Reference: Yao Ming
https://www.basketball-reference.com/players/m/mingya01.html
17. Basketball Reference: Dwight Howard
https://www.basketball-reference.com/players/h/howardw01.html
18. Basketball Reference: Marc Gasol
https://www.basketball-reference.com/players/g/gasolma01.html
19. Basketball Reference: Brook Lopez
https://www.basketball-reference.com/players/l/lopezbr01.html
20. Basketball Reference: DeMarcus Cousins
https://www.basketball-reference.com/players/c/couside01.html

Peer-reviewed sports-medicine literature

21. Season-ending injury risk factors in NBA players
https://pubmed.ncbi.nlm.nih.gov/38311270/

22. NBA injury epidemiology study
https://journals.plos.org/plosone/article?id=10.1371/journal.pone.0263354

23. Load management and subsequent injury study
https://link.springer.com/article/10.1007/s40279-026-02457-w

Source-integrity note

The report preserves the supplied analysis's findings and citations. It does not claim that every linked source supports every adjacent detail beyond the specific proposition for which it is cited. The comparable-center prior, ensemble weights, exposure scaling, and sensitivity adjustments are analyst calculations rather than published medical statistics.

Document metadata and verification

Generation timestamp	July 18, 2026, 11:15:29 PM UTC
Document version	1.0
Analysis certainty score	61 / 100
Hallucination-risk rating	Low for cited historical facts; moderate for the forecast.
Independent review	All calculations and source interpretations should be independently reviewed.
SHA-256 note	The finalized file hash is supplied alongside delivery. It is not embedded here because embedding the final hash would alter the file and therefore change the hash.

Calculation summary

Central significant-injury estimate: 68%. Reasonable model-sensitivity interval: 55%-80%. Estimated probability of at least 30 injury-missed games: 31%.

Report classification: public-data sports-risk analysis. Not a medical diagnosis, betting recommendation, or statement about confidential health information.